Suspected Alcohol Intoxication

Routine Primary and Secondary Assessments
Including:
- Vital Signs
- Blood Sugar
- and 12-lead ECG (if patient is compliant)

Manage Airway, Breathing and Circulation
as required

Consider Transportation

Patient Refuses Transport

Decision Making
Capacity Assessment

Failed Capacity Assessment
and
Patient still refusing transport

Sober, adult caregiver
willing to accept responsibility for the patient

Provide caregiver with care instructions

No Available Caregiver

Request Police Assistance

Transport Decision Making

Transport to hospital should be recommended if **any** of the following are present:
- Abnormal vital signs
- Altered Mental Status
- Hyperglycemia or hypoglycemia
- Evidence of trauma or injury
- History of head injury
- Suspicion of co-ingestion of drugs
- Persistent vomiting
- Seizures or respiratory distress.
- No sober, adult caregiver available to provide care for the patient.

Capacity Assessment

If **all** the below criteria are met, the patient has capacity to refuse transport. **See discussion section.*

- Understanding
- Appreciation
- Reasoning
- Confirmed Expression

Police

No unconscious patient may be left in the care of the police.

Patient Care Goals

1. Identify and manage patients with suspected alcohol intoxication to prevent complications such as airway compromise, hypoglycemia.

Patient Inclusion & Exclusion Criteria

Inclusion Criteria

- Patients presenting with signs and symptoms suggestive of alcohol intoxication, including:
  - Slurred speech, impaired coordination, confusion, or altered mental status.
  - Odor of alcohol on breath (acetone breath) or history of recent alcohol consumption.
  - Behavioral changes (e.g., agitation, aggression, or lethargy).
  - Associated symptoms such as nausea, vomiting, or unresponsiveness.

Exclusion Criteria

- Symptoms clearly attributable to another cause (e.g., head trauma, stroke, or drug overdose) without evidence of alcohol involvement.
- Patients with no clinical signs or history suggestive of alcohol intoxication.

Key Patient Assessment Points

1. **Alcohol Consumption History**
   - Confirm recent alcohol use (e.g., quantity, type, and timing of consumption).
   - Inquire about chronic alcohol use or history of alcohol dependence (e.g., withdrawal symptoms, prior detox admissions).
   - Assess for co-ingestion of other substances (e.g., drugs, medications).
2. **Neurological and Behavioral Assessment**
   - Evaluate level of intoxication using AVPU (Alert, Verbal, Painful, Unresponsive) or Glasgow Coma Scale (GCS).
   - Assess for signs of severe intoxication (e.g., unresponsiveness, respiratory depression, or inability to protect airway).
   - Note behavioral indicators (e.g., aggression, confusion, or disinhibition).
3. **Glucose and Airway Assessment**
   - Check random blood sugar (RBS) to rule out hypoglycemia (<70 mg/dL) or hyperglycemia (>250mg/dL), which can mimic intoxication.
   - Assess for airway compromise (e.g., vomiting with risk of aspiration, depressed gag reflex).
4. **Environmental and Contextual Clues**
   - Look for evidence of alcohol use (e.g., empty bottles, bar setting).
   - Identify potential injuries or trauma sustained while intoxicated (e.g., falls, lacerations).
5. **Investigation of Other Causes**
   - Utilize the AEIOU TIPS pneumonic to investigate alterative causes.
     - A – Alcohol, Abuse, Atypical migraine
     - E – Epilepsy, Electrolytes
     - I – Insulin (hypoglycemia)
     - O – Oxygen, Overdose
     - U – Uremia (kidney failure)
     - T – Trauma, Tumor
     - I – Infection
     - P – Psych, Poisoning
     - S – Seizure, Subarachnoid hemorrhage, Sepsis
6. **Determine Triage Category**
   - Determine Triage Category as per Patient Triage Categories CPG.

Patient Management & Interventions

1. **Primary & Secondary Survey**
   - Perform primary and Secondary surveys as per Universal Care CPG.

2. **Consider Red Flags**
   - Unresponsiveness or coma (AVPU: U).
   - Respiratory depression (RR <10 breaths/min) or hypoxia (SpO2 <90%).
   - Hypotension (SBP <90 mmHg in adults, or age-specific for pediatrics), tachycardia, or poor perfusion (e.g., cool, clammy skin, delayed capillary refill >2 seconds).
   - Hypoglycemia (RBS <70 mg/dL) or severe hyperglycemia.
   - Suspected head injury or trauma sustained while intoxicated.
   - Signs of alcohol withdrawal (e.g., tremors, seizures, delirium tremens).
3. **Consider ALS back-up**
   - Consider the requirement for ALS back-up as per Universal Care CPG.
4. **Consider the requirement for transfer to hospital**
   - All unconscious/unresponsive patients should be transported to a hospital.
   - Many medical conditions can mimic alcohol intoxication which can only be ruled out in hospital.
   - A strong recommendation of transportation to hospital should be made for patients who meet any of the criteria listed below.
   - Transfer to hospital should be recommended if any of the following are present:
     - Abnormal vital signs
     - Altered Mental Status
     - Hyperglycemia or hypoglycemia
     - Evidence of trauma or injury
     - History of head injury
     - Suspicion of co-ingestion of drugs
     - Persistent vomiting
     - Seizures or respiratory distress.
     - No sober, adult caregiver available to provide care for the patient.
5. **Conduct a Capacity Assessment**
   - For patients who are refusing transfer, an assessment of capacity is required to ascertain if the patient is competent to make an informed decision.
   - All these must be met to be deemed capable.
     - Check Understanding: Explain the situation and risks simply (e.g., “You could get worse, we suggest hospital”) then ask them to repeat it back.
     - Test Appreciation: Ask if they see their own risk (e.g., “Why do you think you are okay now?”) to confirm they grasp their condition.
     - Assess Reasoning: Ask them to weigh options (e.g., “What happens if you stay? What’s your plan?”) and look for logical thinking.
     - Confirm Expression: Ask for their decision (e.g., “Do you want to go to hospital?”) and ensure it’s clear and consistent when re-asked after a minute.

Patient Safety Considerations

- No unconscious/unresponsive patient may be left in the care of police.

Clinical Insights & Additional Guidance

- Leaving the Patient with a Caregiver:
  - Patients without capacity to refuse transport must be left with a sober, adult caregiver willing to accept all responsibility for the patient.
  - Provide the caregiver these instructions and ensure their understanding:
    - Always Stay with the Patient: “Do not leave them alone, even for a short time. Stay close and keep an eye on them until they’re fully awake and sober.”
    - Position Them Safely: “Place them on their side with a pillow or cushion behind their back, not flat on their back, especially if they’re sleepy or vomiting.”

- Watch for Breathing Problems: “Check their breathing every few minutes. If it slows (less than 10 breaths a minute), gets noisy, or stops, call 999 immediately.”
- Monitor for Vomiting: “If they vomit, keep them on their side and clear their mouth if you can. Call 999 if vomiting doesn’t stop or they choke.”
- Look for Signs of Worsening: “Call 999 if they won’t wake up, act confused, have a seizure, or seem very sick (e.g., pale, sweaty, cold).”
- Avoid Giving Food or Drinks: “Don’t give them food, water, coffee, or more alcohol. It could make them choke or get worse.”
- Know When to Call for Help: “Call 999 if you’re worried at all—better safe than sorry.”
- Ensure a Safe Environment: “Keep them in a warm, quiet place away from hazards like stairs or traffic. Don’t let them drive or wander off.”

Documentation

- Identification: Patient’s full name, age, and gender.
- Mechanism / Medical Complaint: Suspected or confirmed alcohol intoxication, including behavioral and physical symptoms.
- Injuries / Information: Head trauma, falls, vomiting, seizures, or known medical conditions.
- Signs: Vital signs, GCS or AVPU score, blood glucose level, evidence of trauma, and response to verbal or painful stimuli.
- Treatment:
  - Oxygen therapy: flow rate and method.
  - Blood glucose testing and correction if hypoglycemic.
  - Positioning and airway support as required.
- Allergies: If known or documented in past records.
- Medications: History of alcohol or drug use, prescribed medications.
- Background: Psychiatric history, chronic illness, or prior intoxication episodes.
- Other:
  - Patients with suspected alcohol intoxication should be documented as “acetone breath” unless alcohol intoxication has already been confirmed through blood testing.
  - For patients left in the care of a caregiver: the risks of not being transported to hospital should be explained, and the caregiver must sign the Patient Care Report.

Key Performance Indicators

1. Perform a capacity on all suspected alcohol intoxication patients who are refusing transfer.
2. Transfer all patients who are unresponsive.
3. Perform and document an RBS for all patients with suspected alcohol intoxication.

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Trauma